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Project #54: An Individualized Unit Based Approach to Utilizing Pressure Injury Prevention Strategies for Acute Care Nursing Staff

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AIM

- Hospital Acquired Pressure Injuries (HAPI) are a complex and important safety issue. They increase length of stay, readmission rates, costs, and mortality.
- Per NPIAP Pressure Injury (PI) Fact Sheet- 2019:
 - Avg cost per patient for HAPIs : \$29,000-\$151,700
 - 2.5 million patients develop PI and 60,000 die annually
- At HFH Detroit, HAPI rates from Jan to Aug 2022 peaked to **6.5%** (NDNQI* mean 2021 Q1 benchmark is 2.9%)
- Increased HAPI rate, high staff turnover, and supply chain challenges made evaluating PI prevention strategy implementation crucial.
- HFH Detroit Wound Care Team sought to decrease HAPI rate to the goal of 4% in 12 months.**

APPROACH (PLAN)

- Wound care team developed **an audit tool** to measure compliance of: Current PI Prevention Strategies, Skin Assessment, Documentation on turning/positioning, and Appropriate layering under patients to prevent moisture skin damage
- Audit also identified prevention supplies stocked and available on each unit.
- Data from the audit was used to:
 - Develop education tailored to each unit's needs
 - Address supply stocking issues
 - Identify contributing factors that may increase HAPI rates

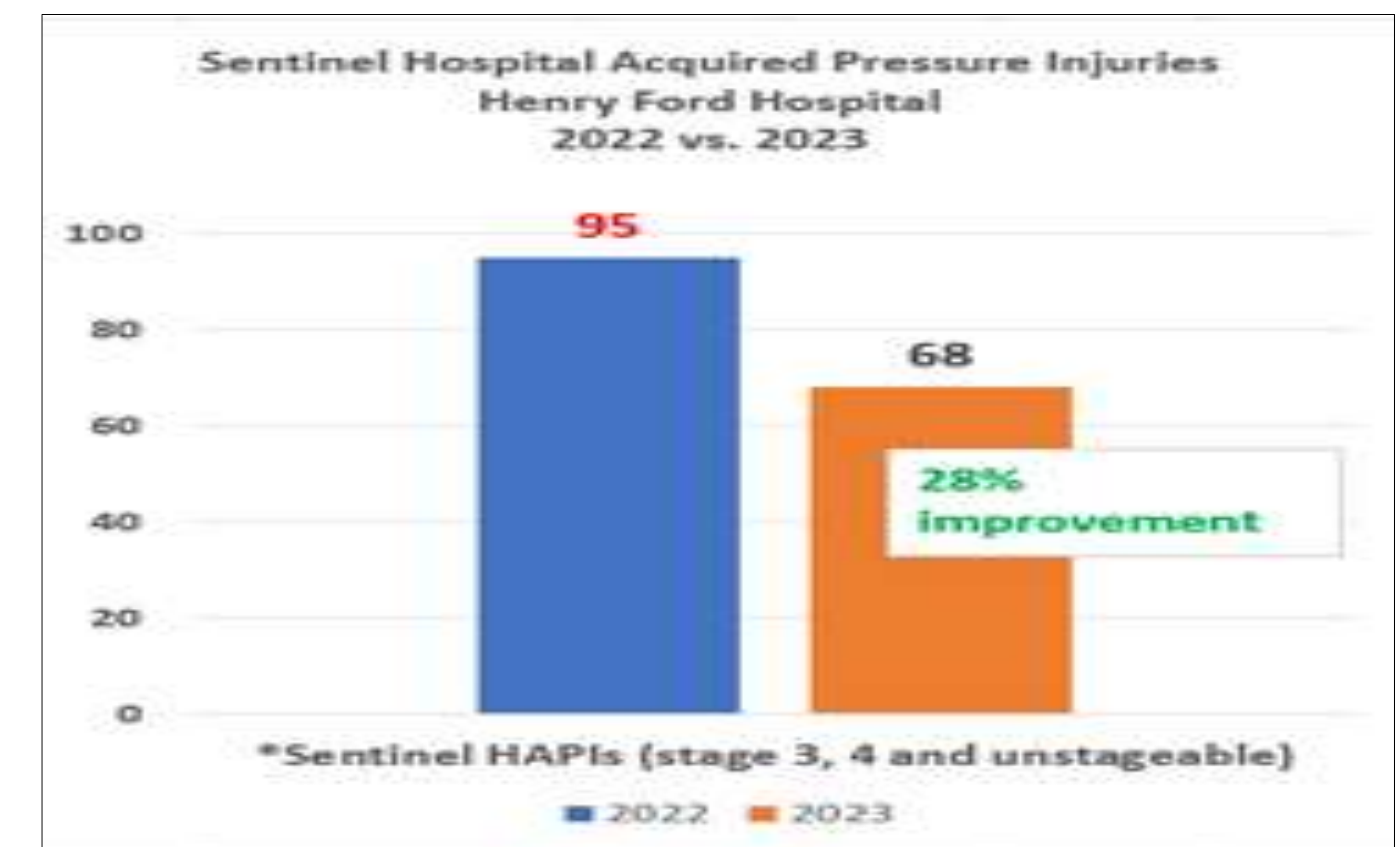
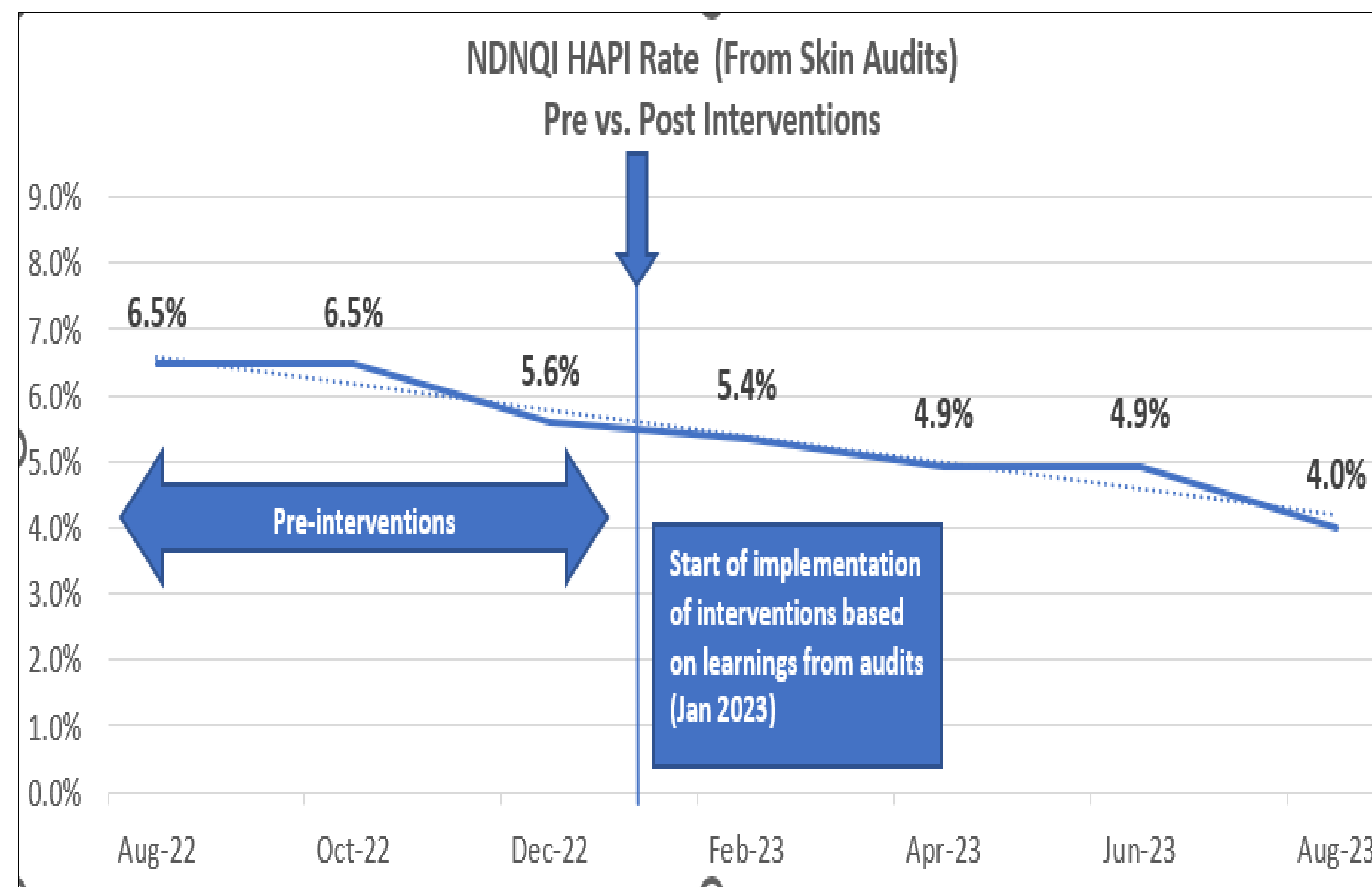
From Aug 2022-Jan 2023, 16 units were audited. Same units were re-audited from Nov 2023-Jan 2024.

INTERVENTIONS (DO)

Multiple interventions were carried out from Jan-Nov 2023 to increase staff knowledge, improve supply availability & increase staff engagement in pressure injury prevention.

- Standardize stock:** Par lists, contacting supplies, etc.
- Visual Controls/Quick references:** badge buddy, laminating toolkit and posting, pamphlet for new hire, QR code for wound toolkit and PI
- Unit Based Education:** Putting together general educational tools for units, tapping into nursing orientation if possible, utilizing skin champions, case studies
- Wound Care Mentor Program**
- Prevention device new product initiation**
- Annual mandatory online nursing education program**
- RN shadowing during orientation**
- Hospital wide education for RNs and NAs**
- Shadowing program for new nurses**

RESULTS (CHECK)



- 38% improvement** in HAPI Rate from Aug 2022 to Aug 2023 (Reduced to 4% from 6.5%)
- 28% improvement** in Sentinel HAPIs (Reduced to 68 in 2023 from 95 in 2022 = **27 fewer Sentinel HAPIs in 2023**)
- Per AHRQ (2017) on an average, a hospital incurs an additional cost of \$14,506 when caring for the patient with a HAPI
- 27 fewer SE HAPIs in 2023 that's an estimated
- Cost avoidance of \$ 391,662** in 2023 over 2022.
- 2- person skin assessment completion compliance improved **by 8%**.
- An **increase of 8% to 18% compliance** in using prevention devices
- 6% reduction** in patients with no interventions in place
- 8% increase in improvement** in using appropriate layering of 1-2 layers under patients which can contribute to moisture skin damage

SUSTAINMENT (ACT)

- Wound care team will continue to provide focused education to units that do not meet the target or has an upward trend.
- Compliance with positioning and turning documentation will be an area of focus to improve upon the 3% improvement.
- Planned education on PIP protocol implementation for observation patients.
- Audit on compliance and usage of Allevyn heel dressings versus offloading boots for PI prevention.
- Specialty beds education for patients at risk due to decrease in ordering

KEYS TO SUCCESS

- Initiating wound care mentoring with unit clinical staff helped in identifying ongoing changes and building engagement with the staff and leadership
- Sharing data results with leadership
- Results solidified the staff need for education regarding pressure identification, assessment and prevention

REFERENCE AIDS: BROCHURES, FLYERS, QR CODES

